

This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction
for the administration of Varivax and Varilrix for vaccination against Chickenpox

Summary of NICE and NICE CKS Guidelines for Chickenpox (Varicella) Vaccination

Vaccine Overview

- The varicella (chickenpox) vaccine is a **live attenuated vaccine**.
- Two licensed brands in the UK: **VARIVAX®** and **VARILRIX®**.
- Typically administered **subcutaneously**, but **intramuscular** administration is also acceptable if necessary.

Indications for Vaccination

The varicella vaccine is recommended for:

1. **Susceptible healthcare workers** and those in contact with immunocompromised individuals (to prevent transmission).
2. **Susceptible close household contacts** of immunocompromised patients.
3. **Non-immune adults and adolescents** (13 years and older) with no history of chickenpox, especially women of childbearing age (with appropriate contraceptive advice).
4. **Susceptible children (over 12 months)** who are at high risk of severe chickenpox or pose a risk to vulnerable contacts.
5. **Outbreak control** in some institutional settings (e.g. schools, nurseries, hospitals).

Note: "Susceptible" = no reliable history of chickenpox or no evidence of immunity.

Dosing Schedule

- **Two doses** are required for full protection:
 - **First dose** at any time from 12 months of age.
 - **Second dose** given at least **4 to 8 weeks later**.
- If given as part of MMRV combination vaccine, different guidance may apply for under-2s (due to febrile seizure risk).

Contraindications

Do **not** administer the vaccine to:

- Individuals with a history of severe allergic reaction to a previous varicella vaccine dose or any component.
- Immunosuppressed individuals (unless under specialist advice).
- Pregnant women (and advise avoiding pregnancy for **at least 1 month** after vaccination).
- People with untreated active tuberculosis or current febrile illness requiring postponement.

Precautions

- Postpone in acute illness with fever.
- Avoid salicylates (e.g. aspirin) for 6 weeks after vaccination due to Reye's syndrome risk.
- Consider prior immunity testing in adults if uncertain.

Vaccine Effectiveness

- Two doses are about **90–95% effective** at preventing chickenpox.
- If breakthrough infection occurs, it is usually mild.

Adverse Effects

- Common: Local site reactions, mild fever, mild rash.
- Rare: Febrile seizures (especially with MMRV in <2 years), varicella-like rash, mild systemic symptoms.

Documentation and Reporting

- Record batch number, site of injection, and date.
- Report any adverse reactions via **Yellow Card Scheme**.

Patient Group Direction

for the supply/administration of

Varivax

for vaccination against

Chickenpox

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	● Pharmacist registered and practicing with the GPhC ● Pharmacy technician registered and practicing with the GPhC
Training and assessment	● Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. ● All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines ● All users must previously have used PGD's to supply medication ● Must work in compliance with the SOPs of their own employer ● All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	● All users must be up to date with CPD requirements and appraisal ● All users must be up to date with MHRA and safety alerts with regards to medical products ● All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Active immunisation against varicella (chickenpox) in individuals aged 12 months and over who are susceptible to the varicella-zoster virus and have no history of chickenpox infection or prior vaccination.
Inclusion criteria	- Individuals aged 12 months and older. - No history of chickenpox infection. - No previous varicella vaccination. - Informed consent given. - Suitable for vaccination in accordance with national immunisation guidelines.
Exclusion criteria	- History of chickenpox infection. - Previous varicella vaccination. - Hypersensitivity to neomycin, gelatin, or any component of the vaccine. - Immunocompromised individuals unless under specialist advice. - Pregnancy or planning pregnancy within one month. - Acute febrile illness or active untreated tuberculosis.
Cautions	- Avoid contact with high-risk individuals (e.g. immunosuppressed) for 4–6 weeks if a rash develops post-vaccination. - Pregnancy must be avoided for one month post-vaccination. - Postpone in the case of moderate or severe illness with fever. - Consider delaying vaccination if the individual received blood products recently.
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate

Details of the medicine

Name, form and strength of medicine	Varivax
Legal category	POM
Storage	Store in a refrigerator (2°C–8°C). Do not freeze. Protect from light.
Route/method of administration	Subcutaneous injection, usually in the upper arm.
Dose and frequency	Single 0.5 mL dose. A second dose may be given after at least 4 weeks if needed.
Quantity to be administered and/or supplied	Single 0.5ml dose
Maximum or minimum treatment period	One or two doses depending on immune status and age group.
Adverse effects	Injection site reactions (pain, redness, swelling), fever, rash, irritability, or upper respiratory symptoms. Rarely, varicella-like rash or febrile convulsions. Refer to SmPC.

Yellow card reporting	Report any suspected adverse effects via https://yellowcard.mhra.gov.uk and notify the GP as appropriate.
Records to be kept	Record the following information: ● that valid informed consent was given ● name of individual, address, date of birth and GP ● name of healthcare practitioner ● name and brand of medication ● date of supply dose, form and route quantity supplied ● advice given, including advice given if excluded or declines treatment ● details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: ● Adults aged 18 years and over - keep records for 8 years ● Children aged under 18 years -keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with the medication.
Follow-up advice to be given to patient or carer	To seek medical advice if symptoms worsen rapidly or significantly, do not improve in 3–4 weeks, or they become systemically very unwell.

Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017 <https://www.nice.org.uk/guidance/mpg2>
- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017 <https://www.nice.org.uk/guidance/mpg2/resources>
- NICE CKS Guidance
- British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines [BNF \(British National Formulary\) | NICE](#)
- Summary of Product Characteristics

Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates			
Name of healthcare professional	Job title	Registration number	Signature
Valid from date		Expiry date	

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or vaccine.

This section must be signed by the superintendent or clinical lead responsible for this service.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Chris Pilkington	Head Pharmacist, Get Real Health Limited (GPhC: 2046322)		19/06/2026

Name of professional group signatory	Job title & name of organisation	Signature	Date
Chris Pilkington	Head Pharmacist, Get Real Health Limited (GPhC: 2046322)		19/06/2026

Signed on behalf of Get Real Health

Name	Qualification	Signature	Date
Nitin Shori	Medical Doctor GMC: 6047293		25/8/2025
Chris Pilkington	Pharmacist GPhC: 2046322		25/8/2025

Change history

Version number	Change details	Date
001	Development & issue of new PGD	25 th June 2025

This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction
for the administration of Varilrix for the treatment of Chickenpox

Patient Group Direction

for the supply/administration of

Varilrix

for vaccination against

Chickenpox

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	● Pharmacist registered and practicing with the GPhC ● Pharmacy technician registered and practicing with the GPhC
Training and assessment	● Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. ● All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines ● All users must previously have used PGD's to supply medication ● Must work in compliance with the SOPs of their own employer ● All users must have appropriate indemnity in place to cover this service
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Cautions	- Avoid contact with high-risk individuals (e.g. immunosuppressed) for 4–6 weeks if a rash develops post-vaccination. - Pregnancy must be avoided for one month post-vaccination. - Postpone in the case of moderate or severe illness with fever. - Consider delaying vaccination if the individual received blood products recently.
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate

Details of the medicine

Name, form and strength of medicine	Varilrix
Legal category	POM
Storage	Store in a refrigerator (2°C–8°C). Do not freeze. Protect from light.
Route/method of administration	Subcutaneous or intramuscular injection.
Dose and frequency	0.5 mL dose. A second dose may be administered at least 6 weeks after the first if required.
Quantity to be administered and/or supplied	0.5ml dose
Maximum or minimum treatment period	One or two doses depending on age and immune status.
Adverse effects	Injection site reactions, fever, rash, irritability. Rare: febrile seizures, varicella-like rash. Refer to SmPC for full adverse effect profile.

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as necessary.
Records to be kept	Record the following information: ● that valid informed consent was given ● name of individual, address, date of birth and GP ● name of healthcare practitioner ● name and brand of medication ● date of supply dose, form and route quantity supplied ● advice given, including advice given if excluded or declines treatment ● details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: ● Adults aged 18 years and over - keep records for 8 years ● Children aged under 18 years -keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

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Change history

Version number	Change details	Date
001	Development & issue of new PGD	25 th June 2025



VACCINE SAFETY REQUIREMENTS, ADDED AT VERSION V002, 9 September 2026

The requirements on the following pages form part of this Patient Group Direction and must be met before any vaccine is administered under it. They were absent from the signed document and were added by this reissue after an estate-wide safety sweep of every vaccination PGD.

This version REPLACES the correction notice that was previously attached to the front of this document. The notice set out these requirements without changing the signed PGD underneath, so a pharmacy printing from its own records, or reading past the first page, still had the original. The requirements are now in the document itself and the notice is withdrawn.

This reissue adds the safety requirements below and changes nothing else. It is NOT a clinical review of this PGD, and one remains outstanding.

Anaphylaxis: what must be in place before you vaccinate

ADRENALINE (EPINEPHRINE) 1 IN 1,000 INJECTION must be immediately available in the room where vaccination takes place, in date, together with a telephone.

A written anaphylaxis protocol consistent with current Resuscitation Council UK guidance must be available, and every person administering must be trained in the recognition and immediate management of anaphylaxis and hold current basic life support training.

Be able to distinguish a faint and a panic attack from anaphylaxis. Green Book chapter 8 sets out the clinical features of each. A faint after an injection is common; anaphylaxis is very rare.

Report any suspected anaphylaxis via the Yellow Card Scheme even where the diagnosis is uncertain.

Observation after vaccination

OBSERVE EVERY PATIENT FOR 15 MINUTES AFTER VACCINATION, SEATED.

Anxiety-related reactions including vasovagal syncope, hyperventilation and transient visual disturbance or paraesthesia can occur before or after any injection, and are commonest in adolescents. They are a response to the needle, not to the vaccine.

Have procedures in place to prevent injury from a faint.

Record that the observation period was completed.

Cold chain and excursions

Store at +2C to +8C in the original packaging to protect from light. Do not freeze, and discard any vaccine that has been frozen.

COLD CHAIN EXCURSION: any stock exposed to conditions outside +2C to +8C must be QUARANTINED and risk assessed in accordance with UKHSA Vaccine Incident Guidance before any further use.

Do not administer excursion stock until that assessment is complete and has been recorded.

Where the product SPC gives specific stability data for a temporary excursion, that data governs; otherwise quarantine and assess.



Disposal

Dispose of used syringes, needles, vials and any unused or reconstituted vaccine in a UN-approved puncture-resistant sharps container.

Follow local authority requirements and Health Technical Memorandum 07-01, Safe management of healthcare waste.

Never re-sheath a needle.

Records: traceability

RECORD THE VACCINE NAME AND BATCH NUMBER for every dose administered. This is a requirement for any biological product and it is what makes a recall actionable.

Record also the expiry date, the dose, the route, the anatomical site, and where more than one vaccine was given at the same visit, the site of each.

Record the name and registration number of the person administering, and that the vaccine was given under this PGD.

Consent in children and young people

Where the individual is UNDER 16, valid consent must come from a person with PARENTAL RESPONSIBILITY, or from the young person where you assess them as GILICK COMPETENT.

Record which of the two applied. Where consent was given by a person with parental responsibility, record their name and relationship to the patient. Where the young person consented on the basis of Gillick competence, record the basis of that assessment.

A parent accompanying a child does not automatically hold parental responsibility. Ask.

Users must be competent in assessing consent in children and young people before working under this PGD.

Version and change record

Version: v002

Issued: 9 September 2026

Supersedes: the previous signed version of this document.

Change: the vaccine safety requirements above were added following an estate-wide sweep of every vaccination PGD, which found that the signed documents were missing, between them, any requirement for adrenaline to be available, any anaphylaxis provision, a stated observation period, a cold chain excursion procedure, a sharps disposal provision, a batch number requirement, and any wording on consent in children and young people. The specific items added to THIS document are: Anaphylaxis: what must be in place before you vaccinate, Observation after vaccination, Cold chain and excursions, Disposal, Records: traceability, Consent in children and young people.

Authorised by Nitin Shori, Medical Director (GMC 6047293) and Chris Pilkington, Head Pharmacist (GPhC 2046322), on 9 September 2026. Signatures applied digitally on their joint instruction, which is the established process for Get Real Health PGDs.